EVOFIT MEALS BUSINESS VAULT

Dietary Consultation Scripts

How to Discuss Nutrition Without Crossing Scope — From Allergies to Emotional Eating

EvoFit Meals

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How to Use These Scripts

These are real conversation frameworks for the twelve nutrition discussions you'll have most often with clients. Each script includes the context (when you'll need it), the conversation itself (what to say), and notes (what to watch for and where the boundaries are).

Don't memorize these word-for-word. Read them, understand the structure, and then practice delivering the key points in your own voice. The goal is to sound like you — not like you're reading from a playbook.

Every script includes scope-of-practice guidance. Personal trainers can provide general nutrition education and customized meal planning. They cannot diagnose medical conditions, prescribe therapeutic diets, or provide clinical nutrition therapy. These scripts are designed to keep you firmly on the right side of that line.

Script 1: Explaining Macros to Beginners

Context

A new client has never heard of macros — or has heard the word but doesn't know what it means. They might be confused by the numbers on their meal plan. They might have seen "IIFYM" on Instagram and want to know what it's about. Or they simply asked, "What are macros and why do they matter?"

This is one of the most common conversations you'll have. Get it right and the client understands their plan at a deeper level, follows it more consistently, and asks smarter questions going forward.

The Conversation

Open with a simple frame:

> **YOU**
> "Great question. Macros is short for macronutrients. There are three of them, and everything you eat is made up of some combination of the three. Think of them like building materials for your body."

Explain each one simply:

> **YOU**
> "The first is protein. Protein builds and repairs your muscles, supports your immune system, and keeps you feeling full. When you're training, protein is the most important macro to get right. Good sources are things like chicken, fish, eggs, Greek yoghurt, and lean beef."

> **YOU**
> "The second is carbohydrates. Carbs are your body's primary energy source — they're what fuels your workouts and your brain. Carbs are in things like rice, potatoes, oats, fruit, bread, and pasta. They're not the enemy — they're your fuel."

> **YOU**
> "The third is fats. Healthy fats support your hormones, your brain function, and your joints. They're in things like nuts, avocados, olive oil, and salmon. You need fats — the key is choosing quality sources and not overdoing the quantity."

Connect it to their plan:

> **YOU**
> "On your meal plan, you'll see each meal has a macro breakdown — grams of protein, carbs, and fats. I've calculated these specifically for your goals. Your protein is set at [X]g per day because [reason — e.g., you're in a muscle-building phase]. Your carbs are at [X]g because [reason — e.g., you're training 4 days a week and need the fuel]. And your fats are at [X]g to keep your hormones and recovery on track."

Make it practical:

> **YOU**
> "You don't need to obsess over hitting exact numbers. Think of it as a target zone — within 10-15 grams either way on each macro is perfectly fine. The goal is consistency over time, not perfection on any

Script 2: Handling "I Hate Meal Prep"

Context

Your client received their meal plan, looked at the recipes, and immediately felt overwhelmed. "I hate cooking." "I don't have time for meal prep." "This looks like a lot of work." This is one of the most common barriers to adherence, and your response determines whether the client follows the plan or abandons it.

The key insight: the client isn't actually saying they hate cooking. They're saying the plan feels like too much work for their current life. Your job is to reduce the perceived effort, not lecture them on the importance of preparation.

The Conversation

Validate first. Don't dismiss.

> **YOU**
> "I hear you, and you're not alone — most of my clients feel that way at first. The good news is you don't need to be a chef or spend your whole Sunday prepping food. Let me show you what this actually looks like in practice."

Reframe meal prep:

> **YOU**
> "When people hear 'meal prep,' they picture spending 4 hours in the kitchen making 21 meals. That's not what I'm asking you to do. Here's what I mean by prep: spend 60-90 minutes once a week — that's it. In that time, you're going to cook two protein sources, make one batch of carbs, and chop your vegetables. That gives you 80% of what you need for the week."

Give them the minimal viable prep:

> **YOU**
> "Here's the simplest version: on Sunday, cook a batch of chicken and a batch of rice. That covers lunches for the week. For dinners, your plan includes 3-4 recipes that take 15-20 minutes to make fresh. Breakfasts are things like overnight oats or eggs — five minutes max. That's it. You're spending about an hour on Sunday and 15-20 minutes on dinner each night."

Offer alternatives if they still resist:

> **YOU**
> "If even that feels like too much right now, let's start smaller. Follow the plan for just 3 days this week. Prep those 3 days only. See how it goes. Once you see how simple the recipes actually are, most people realise it's way less work than they imagined. And if there are specific recipes that feel complicated, tell me and I'll swap them for simpler ones."

Address the time objection directly:

> **YOU**
> "Can I ask — how much time are you currently spending on food each day? Between deciding what to eat, ordering takeout, waiting for delivery, or standing in front of the fridge? Most people spend more time being indecisive about food than they would following a plan. The plan actually saves time

Script 3: Scope Boundaries — What You Can and Can't Advise On

Context

A client asks you something that falls outside your scope as a personal trainer. Maybe they want specific advice about managing their diabetes through diet, or they're asking about an eating disorder, or they want you to interpret blood work results. This happens to every trainer, and how you handle it defines your professionalism.

The goal is to be honest about your boundaries without making the client feel rejected or dismissed. You want them to get the right help while maintaining their trust in you.

The Conversation

When a client asks about managing a medical condition through diet:

YOU

"That's a really important question and I'm glad you brought it up. Here's where I want to be straight with you: as a personal trainer, I can build you meal plans for general health and fitness goals. But when it comes to managing [their condition] through diet, that's something that really needs a registered dietitian or your doctor to weigh in on. They can prescribe a dietary protocol specifically for [condition], and I can build your meal plan around their recommendations."

YOU

"What I'd love to do is this: get a clear directive from your healthcare provider about what your dietary parameters should be — what to include, what to avoid, any specific nutrient targets. Once I have that, I can incorporate those guidelines into your meal plan so everything works together. That way you get the medical expertise where you need it, and the practical meal planning where I can help. Does that work?"

When a client brings up disordered eating patterns:

YOU

"Thank you for trusting me with that. I want to make sure you get the right kind of support here. What you're describing sounds like something that would really benefit from working with a professional who specialises in eating behaviours — a therapist or counselor who focuses on this area. I'm not qualified to guide you through that, and I'd be doing you a disservice if I tried."

YOU

"What I can do is continue to provide you with a balanced, non-restrictive meal plan that supports your overall health. And if you start working with a specialist, I'll happily coordinate your meal plan with whatever approach they recommend. Would you like me to look into some referral options for you?"

When a client asks about interpreting blood work:

YOU

"I can see why you'd want to understand what those numbers mean for your nutrition. The honest answer is that interpreting blood work is outside my lane — that's your doctor's job. What I'd suggest is: take your results to your next appointment and ask your doctor specifically if there are any dietary

Script 4: Handling Supplement Questions

Context

Every client will eventually ask about supplements. "Should I take protein powder?" "What about creatine?" "My friend takes fat burners, should I?" "I saw this supplement on TikTok..."

Supplements are a minefield of misinformation. Your role is to provide honest, evidence-based guidance on common fitness supplements while steering clients away from snake oil and always deferring to their healthcare provider for anything medical.

The Conversation

The universal supplement framework (use this every time):

> **YOU**
> "Supplements are exactly what the name implies — they supplement your diet. They don't replace good nutrition, and no supplement will fix a bad diet. Let's make sure your meal plan and your overall nutrition are solid first, and then we can talk about what might give you an additional edge."

When asked about protein powder:

> **YOU**
> "Protein powder is one of the few supplements I'd call genuinely useful for most people. It's convenient, it's an efficient way to hit your protein target, and it's well-researched. That said, it's just food — it's dried protein in a convenient form. If you can hit your daily protein target from whole foods alone, you don't need it. If you're falling short, a scoop of whey or plant-based protein in a shake is a simple fix."

> **YOU**
> "In your plan, I've included protein powder in [specific meal/snack]. If you'd prefer to get that protein from whole food instead, I can swap it out. Either way works."

When asked about creatine:

> **YOU**
> "Creatine is the most studied supplement in sports nutrition, and the evidence is strong. It supports strength, power output, and recovery. For most people who are strength training, 3-5 grams of creatine monohydrate per day is a reasonable addition. It's affordable and well-tolerated."

> **YOU**
> "That said — if you have any kidney concerns or medical conditions, check with your doctor first. And know that creatine causes a small amount of water retention, which can show up as a 1-2 pound increase on the scale. That's not fat — it's water in your muscles."

When asked about fat burners:

> **YOU**
> "I'm going to be straight with you: most fat burners are overpriced caffeine pills with marketing behind them. The ones that do contain active ingredients often have side effects — elevated heart rate,

Script 5: Nutrition Goal-Setting Conversations

Context

A new client sits down for their nutrition consultation or first check-in. You need to help them set nutrition goals that are specific, realistic, and connected to their deeper motivations. Most clients come in with vague goals: "eat better," "lose weight," "be healthier." Your job is to turn those into concrete targets they can track and achieve.

This conversation also sets the tone for the entire coaching relationship. If you let them keep a vague goal, you'll both be directionless. If you push too hard for specificity, they'll feel interrogated. The sweet spot is curiosity and clarity.

The Conversation

Start with their words, not yours:

> **YOU**
> "Before we build your plan, I want to understand what success looks like for you. If we work together for the next 3 months and everything goes perfectly — what's different about your life? Don't give me the 'right' answer — give me the real one."

Let them talk. Then probe deeper:

If they say "I want to lose weight":

> **YOU**
> "Okay, weight loss is the goal. Let's get specific. How much weight are you thinking? And more importantly — what does that weight loss mean to you? Is it about how you look, how you feel, fitting into certain clothes, having more energy, getting healthier numbers at the doctor? What's the real reason behind the number?"

If they say "I want to eat healthier":

> **YOU**
> "I love that. What does 'healthier' mean to you specifically? Is it more vegetables? Less processed food? Less takeout? Better energy throughout the day? More structured meals? When you picture 'eating healthy,' what does a typical day look like in your mind?"

If they say "I want more energy":

> **YOU**
> "Let's talk about that. When does your energy drop the most? Is it morning, mid-afternoon, after meals? And what does your current eating look like around those energy crashes? Often the energy issue is directly tied to meal timing and what you're eating at specific points in the day."

Turn vague into specific:

Once you have their real motivation, help them set a SMART nutrition goal:

> **YOU**
> "Based on what you've told me, here's what I'd suggest as your primary nutrition goal for the next 90 days:"

Script 6: Emotional Eating — When to Refer

Context

A client tells you they eat when they're stressed, bored, sad, or anxious. They might say things like "I'm fine all day and then I binge at night" or "When I'm stressed, food is the only thing that calms me down" or "I eat my feelings." This is one of the most sensitive conversations you'll have, and it sits right on the boundary between coaching and therapy.

Your role: recognize the pattern, validate the client's experience, provide structural support through the meal plan, and refer to a mental health professional if the pattern is persistent or severe. You are not a therapist. You cannot treat emotional eating. But you can be the person who helps them get the right support while keeping their nutrition on track.

The Conversation

When a client first mentions emotional eating:

> **YOU**
> "Thanks for sharing that with me. A lot of people experience this, and there's no shame in it. Food is comforting — that's not a character flaw, it's biology. Can I ask a few questions to understand the pattern better?"

Questions to assess severity (ask gently):

- ☐ **1.** "How often does this happen — daily, a few times a week, once in a while?"
- ☐ **2.** "Is it usually triggered by something specific — stress, boredom, loneliness, a particular time of day?"
- ☐ **3.** "After it happens, how do you feel? Guilty? Physically uncomfortable? Neutral?"
- ☐ **4.** "Has this been a pattern for a long time, or is it more recent?"

If the pattern is occasional and situational (stress-eating a few times a month):

> **YOU**
> "What you're describing sounds like stress-related eating, and it's really common. Here's what we can do from a nutrition standpoint: I'll build your plan with a structured evening snack that's satisfying enough to reduce the urge to graze. We'll also make sure your daytime meals are substantial enough that you're not arriving at the evening physiologically hungry — because hunger amplifies emotional eating. We won't try to eliminate the habit overnight. We'll build a structure that reduces the triggers."

If the pattern is frequent, intense, or accompanied by guilt/shame:

> **YOU**
> "I appreciate you being open with me about this. What you're describing sounds like it's more than just occasional stress-eating — and I want to make sure you get the right support. I'm great at building meal plans, but this part of the picture is really better served by a professional who specializes in the emotional relationship with food. That might be a therapist, a counselor, or a psychologist who focuses on eating behaviors."

> **YOU**

Script 7: Diet Culture Pushback

Context

A client pushes back on your plan using diet-culture logic: "Shouldn't I be cutting carbs?" "I thought fat makes you fat." "My friend lost 20 pounds on keto — why aren't you putting me on keto?" "I read that intermittent fasting is the only way to lose belly fat."

This is not a knowledge gap — it's a belief system. Diet culture has spent decades drilling these messages into people, and your client has absorbed them. Your job is to gently dismantle the myth without making the client feel stupid for believing it.

The Conversation

When they ask about cutting carbs:

> **YOU**
> "I understand why you'd think that — the low-carb message has been everywhere for 20 years. Here's the reality: carbs don't make you fat. Eating more calories than your body uses makes you fat, regardless of where those calories come from. Carbs are your body's preferred fuel source — especially for the kind of training you're doing. When I set your carbs at [X]g, it's because your body needs that fuel to perform, recover, and actually burn fat efficiently. Cutting carbs below what your body needs doesn't speed up fat loss — it slows it down and makes you miserable."

When they insist on a specific diet trend:

> **YOU**
> "I've seen clients get great short-term results on [keto/IF/Whole30/whatever]. The problem is the word 'short-term.' The question isn't whether you'll lose weight on [diet] — you probably will, because it creates a calorie deficit. The question is whether you'll still be following it in 6 months. And the data says: almost certainly not. What we're building together is something you can sustain permanently — not a 30-day sprint followed by a rebound."

When they quote something from social media:

> **YOU**
> "Social media is powerful but it's also unregulated. Anyone can post nutrition advice — there's no credential requirement. What I see a lot is people taking a genuine study, oversimplifying it, and turning it into a rule that sounds compelling but isn't accurate. Can you show me what you read? I'd love to look at it with you and separate the science from the spin."

Notes

- ☐ Never dismiss a client's belief aggressively. "That's wrong" creates defensiveness. "Here's what the research actually shows" creates curiosity.
- ☐ If a client has had genuine success on a specific diet and wants to continue it, you can build their meal plan within that framework — as long as it's nutritionally adequate and sustainable. Keto meal plans, IF schedules, and plant-based plans are all possible within EvoFit Meals.
- ☐ The goal is not to "win" the argument. The goal is to build trust so the client follows your plan. Sometimes that means accommodating their preferences and letting the results speak.

Script 8: Client Resistance to Tracking

Context

You've asked a client to track their food intake — or at least follow the plan portions carefully — and they're pushing back. "I don't want to be obsessive about food." "Tracking makes me anxious." "I just want to eat intuitively." "Counting calories gives me a bad relationship with food."

This is a valid concern, and it needs to be handled with sensitivity. For some clients, tracking is empowering. For others, it triggers anxiety or past disordered eating patterns. Your response should respect their experience while finding a middle ground that still supports progress.

The Conversation

Validate first:

> **YOU**
> "I completely hear you. Tracking isn't for everyone, and I never want nutrition to feel like an obsession or a source of stress. The last thing we want is for this to create anxiety around food."

Explore the concern:

> **YOU**
> "Can you help me understand what specifically feels uncomfortable about tracking? Is it the act of logging meals in an app? The feeling of restriction? A past experience with dieting that made food feel stressful?"

If the concern is mild (they just find it tedious):

> **YOU**
> "Here's the good news: your meal plan already does the tracking for you. Every meal has portions and macros pre-calculated. If you follow the plan as written, you don't need to log anything — the plan IS the tracker. The only time I'd ask you to note something is if you eat something significantly off-plan, just so I can adjust. Is that manageable?"

If the concern is deeper (past disordered eating or anxiety):

> **YOU**
> "Thank you for telling me that. Your mental health around food matters more than any macro target. Here's what I'd suggest: we skip the tracking entirely. Your meal plan has everything portioned out — follow it as closely as feels comfortable. At our check-ins, instead of looking at numbers, we'll talk about how you feel: energy, mood, satiety, and whether your clothes are fitting differently. Results don't require a food diary. They require consistency and structure, which your plan provides."

> **YOU**
> "If at any point the plan itself starts feeling like a source of stress — tell me. We can loosen the structure. A flexible guideline you follow peacefully is worth more than a rigid plan that makes you anxious."

Notes

Script 9: Medical Nutrition Referral — When and How

Context

A client discloses a diagnosed medical condition that affects their diet — or you notice signs that suggest an undiagnosed issue. This could be diabetes, PCOS, thyroid disorders, autoimmune conditions, IBS, Crohn's disease, food allergies confirmed by testing, or kidney disease. Your responsibility is clear: refer to a registered dietitian or physician, offer to coordinate, and continue providing general meal planning within the parameters they set.

The Conversation

When a client discloses a condition at intake:

YOU

"Thank you for sharing that. Having [condition] doesn't mean we can't work together — it just means we need to work smart. Here's my approach: I'm going to recommend that you connect with a registered dietitian who specializes in [condition]. They'll set the dietary parameters — what to include, what to avoid, any specific nutrient targets. Once I have those guidelines, I'll build your meal plan within them."

YOU

"Think of it as a team: the dietitian handles the clinical side, and I handle the practical side — making sure you have actual meals you enjoy that fit within their recommendations. This is the safest and most effective approach for you."

When you notice signs during coaching:

YOU

"[Name], I want to bring something up because I've noticed [specific observation — e.g., you're consistently low energy despite adequate calories, your weight isn't responding the way we'd expect, you mentioned GI symptoms every time you eat certain foods]. I'm not diagnosing anything — that's not my role — but I think it's worth getting checked out by your doctor."

YOU

"If there's something going on that's affecting how your body processes food or uses energy, no amount of meal planning will fix it until we address the underlying cause. Would you be open to scheduling a check-up? I'll adjust your plan based on whatever they find."

The coordination offer:

YOU

"If your doctor or dietitian gives you specific dietary guidelines, send them to me — even a summary is fine. I'll rebuild your plan to incorporate everything they recommend. You won't have to figure out how to translate medical advice into actual meals. That's my job."

Notes

- ☐ Keep a current referral list. Include: 2-3 registered dietitians (ideally with different specializations), a therapist who specializes in eating disorders, and your local GP or sports medicine doctor.

Script 10: Cultural Food Sensitivity

Context

You build a meal plan and the client responds with "I don't eat those foods" or "This isn't how my family eats" or "We have specific foods for certain occasions." Food is deeply cultural, and a meal plan that ignores a client's cultural food traditions won't be followed — no matter how good the macros are.

This script helps you navigate cultural food preferences with respect, curiosity, and practical flexibility.

The Conversation

When a client raises cultural food concerns:

> **YOU**
> "I'm glad you told me. Your food traditions are important, and a meal plan that asks you to eat completely differently from your family or your culture is a meal plan that's going to collect dust. Let me learn about how you eat so I can build around it, not against it."

Ask open-ended questions:

> **YOU**
> "Walk me through a typical family meal in your household. What does dinner usually look like? What are the staple foods your family eats regularly? Are there foods that are important for religious or cultural occasions?"

> **YOU**
> "Are there any dietary rules I should know about? For example, Halal or Kosher requirements, fasting periods, or specific foods for celebrations?"

> **YOU**
> "When you think about 'healthy eating,' does it look different in your mind than what you see on typical fitness accounts? I want to understand your version, not impose mine."

Build the plan around their culture:

> **YOU**
> "Here's what I'm going to do: I'll build your plan using the foods and cooking styles your family already eats as the foundation. If your household eats rice and curry most nights, we're not replacing that with chicken breast and broccoli. We're keeping the rice and curry, adjusting the portions and ingredients to hit your macro targets, and adding variety from there."

> **YOU**
> "The platform has recipes from dozens of cuisines — Mediterranean, South Asian, East Asian, Latin American, Middle Eastern, West African, Caribbean, and more. I'll filter for your preferred cuisines and build your plan around foods that feel like home."

The golden rule:

Script 11: Dealing With Social Media Nutrition Misinformation

Context

A client comes to you with something they saw on TikTok, Instagram, or YouTube. "Is it true that [outrageous claim]?" "This influencer says I should [unsubstantiated advice]." "I watched a documentary that says [misleading conclusion]." Social media nutrition misinformation is relentless, and your clients are exposed to it daily. Your job is to be the trusted filter — not dismissive, not preachy, but factual and calm.

The Conversation

The three-step response framework: Step 1: Don't dismiss. Ask.

> YOU
> "Interesting — what exactly did they say? Can you show me the post or video? I want to see the actual claim before I comment on it."

This does two things: it shows respect for the client's curiosity, and it gives you the actual information to evaluate rather than a telephone-game summary.

Step 2: Evaluate out loud.

> YOU
> "Okay, so they're claiming [specific claim]. Here's how I evaluate nutrition claims: First, is there a credible study behind it — not a single study with 12 participants, but a body of evidence? Second, does the person making the claim have relevant credentials, or are they selling something? Third, does the claim account for individual differences, or is it presented as a universal truth?"

Walk the client through your evaluation. This teaches them critical thinking about nutrition information — which is more valuable than just saying "that's wrong."

Step 3: Give your assessment.

> YOU
> "Based on what I'm seeing, here's my take: [your honest assessment]. [If the claim has merit:] There's actually some truth to this, but the way it's presented oversimplifies it. Here's the nuanced version... [If the claim is misleading:] This is taking a kernel of truth and stretching it beyond what the evidence supports. Here's what the research actually shows... [If the claim is outright false:] I understand why this sounds compelling, but it's not supported by evidence. Here's why..."

Common misinformation topics and quick responses: ClaimQuick Response "Eating after 8 PM makes you fat""Calories don't know what time it is. Total daily intake matters, not meal timing. Eat when it works for your schedule." "You need to detox your body""Your liver and kidneys detox your body 24/7. No juice cleanse does it better. Save your money." "Artificial sweeteners are dangerous""The evidence says they're safe in normal amounts. If you prefer to avoid them, that's fine — but it's a preference, not a medical necessity." "You can't absorb more than 30g of protein per meal""Outdated. Your body absorbs all the protein you eat — what changes is the rate of muscle protein synthesis, and even that threshold is higher than 30g for most people." "Carbs spike

Script 12: Post-Competition Rebound

Context

A client has just finished a physique competition, a dramatic weight cut for a sport, or any extended period of severe dieting. They're physiologically and psychologically primed for a rebound — their hunger hormones are elevated, their metabolic rate is suppressed, and they've been restricting so long that the psychological pressure to eat freely is enormous. How you manage the transition back to normal eating determines whether they maintain their results or gain back everything (and more) within weeks.

The Conversation

Before the competition/event (have this conversation during prep):

> **YOU**
> "[Name], I want to talk about what happens after [competition/event]. The prep phase is extreme by design — the calorie deficit, the precise macros, the food restrictions. That's necessary for the goal. But it's not sustainable, and your body knows it. After the event, we need a structured reverse diet to bring your calories back up gradually without a rebound."

> **YOU**
> "Here's what I've seen happen to people who don't have a plan for after: they finish the competition, they feel amazing, and then the flood gates open. Within 2-3 weeks, they've gained back 10-15 pounds, they feel terrible about it, and they end up worse off than when they started prep. I don't want that for you."

After the competition/event:

> **YOU**
> "First — congratulations. What you just accomplished took incredible discipline. Now the job changes. The goal for the next 4-6 weeks is a structured reverse diet. We're going to bring your calories up gradually — about 100-150 calories per week — to find your new maintenance level without triggering a rebound."

> **YOU**
> "Here's the reverse protocol:"

The Reverse Diet Framework: Week Post-EventCalorie AdjustmentFocus Week 1+100-150 cal above prep end pointIncrease carbs first (refill glycogen). Keep protein stable. Add a rest day to recover. Week 2+100-150 calContinue increasing carbs. Add small amount of dietary fat. Scale weight will increase — this is water and glycogen, not fat. Week 3+100-150 calIntroduce more food variety. Allow 1-2 untracked meals per week. Monitor psychological state. Week 4+100-150 calApproach estimated new maintenance. Hunger hormones should begin normalizing. Week 5-6Stabilize at maintenanceHold calories steady. Monitor weight trends. Adjust if weight gain exceeds 1-2 lbs per week beyond water normalization. **Managing the psychological side:**

> **YOU**

Quick Reference: The Scope Cheat Sheet

Keep this somewhere accessible. When in doubt, check it.

TopicIn Scope (You Can Advise)Out of Scope (Refer Out) Macros and caloriesGeneral targets for fitness goalsTherapeutic diets for medical conditions Meal planningCustom plans for health and body compositionDiets prescribed for diabetes, kidney disease, etc. Food choicesRecommending whole foods, recipe suggestionsEliminating foods to treat allergies or medical conditions SupplementsGeneral fitness supplements (protein, creatine)Supplements to treat medical conditions Weight managementCalorie deficit/surplus strategies for body compositionWeight management for eating disorders HydrationGeneral hydration guidelinesFluid management for medical conditions Meal timingPre/post-workout nutrition, general meal spacingTherapeutic fasting for medical purposes Emotional eatingRecognising patterns and suggesting structureDiagnosing or treating eating disorders Food intolerancesAvoiding reported intolerances in meal plansDiagnosing food allergies or intolerances Children's nutritionGeneral healthy eating guidance for parentsPaediatric dietary therapy Cultural food needsAdapting plans to cultural preferencesReligious dietary counseling Social media claimsEvaluating and explaining evidenceMaking clinical counter-claims Post-competitionReverse dieting and transition plansTreating disordered eating or body dysmorphia Medical referralsRecognizing when to referDiagnosing or treating any condition **The golden rule:** If a client's question starts with "Can this food cure/treat/manage my [medical condition]?" — the answer is always "Let's get your doctor's input on that, and I'll build your plan around their recommendations." *This guide is part of the EvoFit Meals Business Vault. Access your full vault at meals.evofit.io/vault*

EvoFit Meals

Build Your Nutrition Coaching Business

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